

Psychosocial Analysis

Annabelle P. J. Fai

University of South Carolina: College of Social Work

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Scott Fairweather

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Client information

Noah is a 26-year-old, white, transgender man who uses he/him/his pronouns. He identifies as male and has been undergoing gender-affirming care, including the use of testosterone. While Noah's sexual orientation was not explicitly stated, his gender identity plays a significant role in his psychosocial context. He is currently single and lives independently. Noah was baptized and raised in a Christian household, which involved regular church attendance during childhood. However, he has since ceased attending religious services, primarily due to the church's disapproval of his gender identity. Noah is employed as an IT Helpdesk worker at a local university. He was referred for counseling services by his insurance company and is currently seeking individual therapy. His emergency contact is his mother, though the nature of their relationship presents stressors that contribute to his presenting concerns.

Presenting issues

Noah is seeking individual counseling due to heightened, and persistent, anxiety. Over the past six months, he has reported frequent episodes of feeling as though he is "freaking out all the time." His symptoms include chronic insomnia, diminished appetite, and pervasive worry, particularly centered on concerns about his mother, who was recently diagnosed with stage-4 breast cancer. This period of intensified distress appears to be the precipitating event that led Noah to initiate counseling. These concerns have begun to impact his daily functioning, prompting the referral and request for therapeutic intervention. His referral source was his insurance company, and he has consented to the exchange of treatment-related information with both his mother and the physician who prescribes his hormone therapy.

Client History

Identifying and Family Information

Noah's early development appears typical, with no reported prenatal or early childhood complications. His family includes his mother, who remains a central figure in his life, three older sisters, which he no longer has contact with, and a father who left the family when Noah was 2 years old. There is a history of family conflict, which is rooted in differing views on his gender identity. Noah's ties to his mother are strong after navigating tension relating to his gender identity. Recently, his mother's cancer diagnosis has become a major stressor and catalyst for seeking therapy.

Biophysiology

Noah has no known history of chronic physical illness, but is currently prescribed testosterone as part of his gender-affirming hormone therapy. He takes a daily multivitamin and has no reported hospitalizations or significant psychiatric interventions, aside from the psychological evaluation required prior to beginning testosterone therapy. He denies any history of suicidal ideation or attempts

Employment and Finances

Noah holds a position in the IT helpdesk department at a local university. His employment history reflects stability and competence, and he appears to function well in professional settings. He has not reported any significant financial hardship, substance use, or involvement with the criminal justice system. There is no history of military service. He reports no engagement in interpersonal violence, and he has no legal record.

Social

Noah's social life appears limited, and he has identified loneliness as a recurring issue. His engagement in recreation and leisure is limited to primarily work colleagues and playing video games with online friends. This suggests a potential area for further exploration. Social support within the

family is limited, though his protective instincts toward his mother suggest a capacity for deep interpersonal commitment.

Psychosocial

Noah currently lives alone, and there is no indication of housing instability or unsafe environmental conditions. External factors such as his identity as a transgender man, his estrangement from certain religious and familial communities, and the emotional toll of caregiving for a parent with cancer are highly relevant contextual influences.

Assessment

Life Stage and Developmental Analysis

Noah is a 26-year-old adult in early adulthood, a developmental period characterized by increasing independence, identity consolidation, and the formation of long-term relationships and career pathways (Hepworth et al., 2022). According to Erikson's psychosocial stages, Noah is navigating the developmental task of *intimacy vs. isolation*, where forming meaningful emotional connections is a core developmental challenge. Noah's difficulty maintaining a social life outside of work, and his reliance on his mother for emotional support, show the major impacts of his isolation. These difficulties may stem from both his past experiences of marginalization, as a transgender man, and his current stressors of coping with his mother's cancer. His struggle with anxiety, sleep disruption, and reduced appetite may also reflect a more difficult transition through this life stage, which may be further exacerbated by unresolved past trauma and current role strain.

Cognitive, Emotional, and Behavioral Evaluation

Noah's cognitive processes are marked by persistent worry, especially concerning his mother's health. His emotional state includes anxiety, loneliness, and depressive symptoms, which he describes as "freaking out all the time." His behavioral responses—difficulty sleeping, diminished appetite, and

reduced motivation to engage with others—are consistent with clinical symptoms of generalized anxiety disorder and situational depression.

Motivation and Engagement

Despite his emotional challenges, Noah has demonstrated a moderate level of motivation by seeking counseling services on his own. Noah's decision to seek help indicates insight into his deteriorating mental health and a desire to regain emotional stability. However, his engagement may be hindered by low energy, emotional exhaustion, and distrust from past invalidating experiences—which he has already expressed. Supportive and collaborative goal setting will be critical in fostering continued participation in treatment.

External and Contextual Factors

From a person-in-environment (PIE) perspective, Noah's current challenges must be understood within the context of his intersecting identities and environmental stressors. As a transgender, Noah has faced identity-based marginalization from family and others, which has shaped his emotional experiences and limited his access to consistent support. Although he no longer attends church due to its stance on LGBTQ+ identities, early religious teachings and familial norms may still influence his internal conflicts and feelings of guilt or rejection.

He is employed in IT support at a local university, which provides financial stability; however, he is simultaneously supporting his mother, who is undergoing treatment for cancer. This dual role as a caregiver and employee increases his emotional burden and may contribute to chronic stress and anxiety. Noah's current living situation appears stable, but his social support network is limited. Outside of work, he has little engagement with others, and he identifies feelings of loneliness and emotional isolation.

Strengths

Despite the challenges Noah is currently facing, he possesses a range of personal, interpersonal, and contextual strengths that can be leveraged throughout the intervention process. One of Noah's biggest strengths is his self-awareness. He has been able to recognize his increasing distress and take the initiative to seek professional help, which is a clear indicator of insight and a desire for growth or change. His ability to articulate his experiences—such as describing how he is “freaking out all the time” and noticing changes in sleep and appetite—demonstrates emotional literacy and an openness to addressing his mental health.

Noah also shows resilience, developed through navigating complex life experiences including his gender transition and associated psychological evaluations, marginalization from his religious community, and now coping with a parent's serious illness. These experiences, while difficult, suggest that Noah has already developed foundational coping skills that can be built upon in counseling.

Noah's employment at the local university provides stability and a structured routine, and reflects his ability to maintain responsibilities even during periods of emotional strain. Although his social network may be limited, his ongoing relationship with his mother remains a source of emotional connection and motivation.

Noah also benefits from having access to medical and mental health care, including ongoing hormone therapy and the ability to pursue counseling services. His willingness to authorize the release of information to both his doctor and his mother suggests he values collaborative care and has a foundation of trust with at least a few key individuals in his life.

Change-Oriented Approach

Based on Noah's presenting issues, as well as the context of his emotional and environmental stressors, the task-centered approach seems to be the best fit for intervention. As described in *Hepworth et al. (2022)*, task-centered practice is especially effective for clients dealing with situational

stressors and immediate life problems that are impacting daily functioning. This is a short-term, goal-focused approach, that emphasizes problem-solving in a collaborative manner, helping clients build self-efficacy and gain traction in addressing concerns.

Noah's symptoms of persistent anxiety, sleep disturbance, diminished appetite, and emotional distress related to his mother's health are clearly disrupting his ability to function in daily life. Task-centered practice is suited to address these challenges, because it breaks down overwhelming problems into smaller, manageable tasks that can be tackled incrementally. This process can help reduce Noah's sense of helplessness and enhance his sense of control over his environment.

The task-centered approach also fits Noah's situation, given his more moderate motivation. This background may make him wary of prolonged or overly theoretical interventions. A structured, time-limited model that values collaboration and focuses on achievable change is likely to be more engaging and empowering for Noah. Furthermore, it provides flexibility to address both internal emotional struggles and external life stressors, such as his concern for his mother, sleep difficulties, and social isolation.

Intervention Plan

The primary focus of Noah's intervention should be identifying anxiety triggers and reducing related symptoms. A secondary but equally important focus involves strengthening Noah's emotional support system and reducing his reliance solely on his mother for emotional connection. These areas will be addressed through SMART goals that provide a structured and measurable framework for tracking progress and guiding Noah's engagement in the intervention process.

To address anxiety, Noah will begin journaling for approximately two to three weeks, documenting each instance of a panic attack along with relevant context, such as setting, thoughts, and physical symptoms. This reflective practice will assist both Noah and the social worker in identifying specific triggers and patterns. In tandem with journaling, Noah will also begin practicing grounding

techniques, such as deep breathing exercises and meditation, for at least five minutes per day. The effectiveness of these strategies will be monitored through a daily self-assessment, where Noah will rate the severity of his anxiety on a scale from 1 to 10 within his journal. Over time, these ratings will be used to track symptom reduction and evaluate the usefulness of various coping methods.

To reduce Noah's sense of isolation and expand his support network, one goal will be to identify an LGBTQ+ support group that aligns with his needs. With or without the social worker's assistance, Noah will aim to locate and attend a group session within a two-week period. This timeline allows space for both logistical preparation and emotional readiness. Building on this, Noah will be encouraged to engage socially within the group setting by initiating conversation with at least one new person per session over the course of two weeks. These interactions will support the development of new relationships, and help Noah establish a broader base of emotional support beyond his mother. Progress on both goals will be reviewed in biweekly sessions, and adjustments will be made collaboratively to ensure goals remain realistic, responsive, and empowering for Noah as he continues his work in therapy.

References

Hepworth, D.H., Rooney, R.H., Dewberry-Rooney, & G., Strom-Gottfried, K. (2022). *Direct social work practice: Theory and skills (11th ed)*. Brooks/Cole. ISBN: 9780357630594